



POLICY

Patient Rights under the Mental Health Act

Scope (Staff):	All staff
Scope (Area):	CAMHS-wide

Child Safe Organisation Statement of Commitment

CAHS commits to being a child safe organisation by applying the National Principles for Child Safe Organisations. This is a commitment to a strong culture supported by robust policies and procedures to reduce the likelihood of harm to children and young people.

This document should be read in conjunction with this [disclaimer](#)

Only use this box if relevant (e.g. clinical document or available on an external website)

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Aim

To ensure that the rights of children are upheld, as legislated within the [Mental Health Act 2014 \(WA\)](#) (MHA). To describe the requirements that must be met by CAMHS staff with respect to these rights and to provide guidance to staff that are considering restricting, limiting or denying a child any of these rights.

Risk

Quality of care and individual health outcomes may be compromised if the rights of children and their families attending mental health services are not upheld.

Background

This policy explains the rights of children pursuant to the MHA and the obligations of staff with respect to these rights. This policy outlines the rights applicable to all CAMHS services and the additional rights applicable to inpatient CAMHS services. The only CAMHS service that operates as an inpatient facility is CAMHS Ward 5A.

Definitions

Authorised hospital: An authorised hospital is a hospital or part of a hospital authorised under the MHA pursuant to section 541. CAMHS Ward 5A is the only CAMHS service that is classified as an authorised hospital under the MHA.

Carer: A person who provides care or assistance to a person who meets the criteria in s 5(1) of the [Carers Recognition Act 2004](#) (WA). For a child of Aboriginal or Torres Strait Islander descent, this may be a person from their kinship group who is recognised under customary laws and traditions as their carer (MHA s 281(2)(b)).

Child: A person under 18 years of age (MHA s 4).

Family: The person(s) who has parental responsibility for the child. Parental responsibility, in relation to a child, means all the duties, powers, responsibilities and authority which, by law, parents have in relation to children (MHA s 4, [Family Law Act 1975](#) (Cth) (FLA) s 61B and [Family Court Act 1997](#) (WA) (FCA) s 68). For this policy, this also includes any legal guardian of the child appointed under the [Guardianship and Administration Act 1990](#) (WA) (GAA) (GAA s 3). Changes to relationships such as parental separation, divorce or remarriage do not alter parental responsibility, unless there is a specific court order changing parental responsibility (FLA s 61C(2), FCA s 69(2)).

Identified person: An identified person as defined in section 348 of the MHA. The following people constitute an identified person:

- A person who is referred under the MHA for an examination to be conducted by a psychiatrist (MHA ss 26(2), 26(3)(a), 36(2));

- A voluntary inpatient who has been ordered for assessment by a medical practitioner or authorised mental health practitioner under the MHA (MHA s 34(1));
- A person whose detention at an authorised hospital has been extended by a psychiatrist to allow for further psychiatric examination (MHA s 55(1)(c));
- A person who has been ordered by a psychiatrist to be received and detained by an authorised hospital for the purposes of psychiatric examination (MHA s 61(c));
- An involuntary patient;
- A person who is under a hospital order pursuant to section 5(2) of the *Criminal Law (Mentally Impaired Accused) Act 1996* (WA) (MIA);
- A person who is classified as a mentally impaired accused and has been detained at an authorised hospital pursuant to the MIA;
- A person who is classified as a mentally impaired accused and has been released on a release order pursuant to the MIA (MIA ss 35(1), 35(4));
- A person who is a resident of a private psychiatric hostel pursuant to the *Private Hospitals and Health Services Act 1927* (WA) (PHHS) (PHHS s 26P);
- A person who has or may have a mental illness and is being provided with treatment or care by a body or organisation prescribed by the regulations for this paragraph;
- A voluntary patient that is deemed by the Minister to fall into a class of identified persons under section 354 of the MHA.

Involuntary patient: An involuntary patient is a person who is under an involuntary treatment order (MHA s 21(1)). An involuntary treatment order is an inpatient treatment order or a community treatment order (MHA s 21(2)). For more information regarding involuntary treatment orders please refer to the MHA (MHA ss 22(1), 23(1)).

Involuntary inpatient: An involuntary patient who is under an inpatient treatment order.

Management plan: Recovery-informed plan, which includes clear identification of issues, shared goals, actions and timeframes developed in collaboration with the child and family.

Mentally impaired accused: A person who has been charged with an offence and is under a current custody order that they have not been discharged from ([Criminal Law \(Mentally Impaired Accused\) Act 1996](#) (WA) s 23).

Nominated person: The person nominated under section 273(1) of the MHA. The role of a nominated person is to assist the child by making sure that the child's rights and interests are upheld. Nominated persons can be a friend or relative of the child or a child's guardian. It is whoever the child feels should receive information and who can

help the child. The person nominated must be an adult over the age of 18. The formal requirements for a valid nomination are outlined in section 275 of the MHA.

Personal possessions: Chattels as outlined in section 259(2) of the MHA. These include:

- Articles of clothing, jewellery or footwear belonging to the patient;
- Articles for personal use by the patient;
- Aids for daily living, or medical prostheses, that are usually used by the patient as a means of assistance or to maintain the patient's dignity.

Voluntary patient: A voluntary patient is a person who is being provided with treatment or is proposed to be provided with treatment by a mental health service (MHA s 4).

Voluntary inpatient: A voluntary patient who has been admitted to a mental health service as an inpatient (MHA s 4).

Principles

- CAMHS staff must have regard to the [Charter of Mental Health Care Principles](#) (the Charter). The Charter relates to child and family-centred care, the recovery approach and the involvement of families.
- Children and families must always be treated with respect and dignity.
- Children and families must be involved in treatment, care and recovery planning.
- Staff must promote the right of children to seek advocacy and feedback.
- The [Speaking Up for Safety](#) CODE and the [Promoting Professional Accountability](#) reporting tool are relevant to the rights of children.

The Charter

CAMHS staff must read and understand the [Charter](#) and make every effort to operate in accordance with these principles.

Patient Rights that may be Invoked in any CAMHS Service

The Right to an Explanation of Rights

The child's rights must be explained to both the child and their family (MHA ss 244, 245). These rights must be explained:

- As soon as practicable;
- Both verbally and in writing ([Mental Health Regulations 2015 \(WA\)](#) (MHR) r 10(2)); and

- Using language that the child and their family can understand (MHA s 9(2), MHR r 10(2)).

These rights must be repeated to the child and their family a second time at the next appropriate opportunity.

Where the child does not have any family present, these rights must be explained to the child's carer, nominated person or other personal support persons (MHA s 245(1)). This can occur without the consent of the child or their family (MHA s 245(2)). Other personal support persons may include close family members that do not have parental responsibility for the child.

Consideration must be given to the language, developmental stage and literacy levels of the child and their family. MHA [resources](#) can be employed to assist with communicating these rights. It may be appropriate to engage an interpreter to assist with communicating these rights. For more information please refer to the [CAHS Language Services Policy](#).

The clinician that explained these rights must document this interaction in the child's medical record. Documentation is necessary as this can constitute a measure of compliance undertaken by the Chief Psychiatrist.

For further guidance, please refer to the following resources:

- [MHA](#) ss 243 - 246
- [Clinicians' Practice Guide to the Mental Health Act 2014](#) (CPG) Chapter 1 (1.5)

When to Explain These Rights

CAMHS Community Based Services and Outpatient Services

These rights must be explained to the child and their family when:

- an inpatient treatment order is made (the rights must be explained by the psychiatrist who made the order) (MHA s 243(b));
- a community treatment order (CTO) is made (the rights must be explained by the psychiatrist who made the order) (MHA s 243(d)); and
- a child is referred for examination by a psychiatrist (the rights must be explained by the medical practitioner who made the referral) (MHA s 243(f), [Form 1A Referral for Examination by a Psychiatrist](#)).

CAMHS Ward 5A

These rights must be explained to the child and their family when:

- a child is referred for examination by a psychiatrist (the rights must be explained by the medical practitioner or mental health practitioner who made the referral) (MHA s 243(e), [Form 1A Referral for Examination by a Psychiatrist](#));
- a child has been placed under and an Inpatient Treatment Order (MHA s 21(2)). The rights must be explained by the treating psychiatrist or a delegate.

- a child under an inpatient treatment order is granted leave from the hospital by a psychiatrist pursuant to section 105 of the MHA (MHA s 243(c));
- a child is admitted as an inpatient (this must occur at admission and the rights are explained by the nursing staff) (MHA s 243(a)); and
- a CTO is made on discharge (the rights must be explained by the psychiatrist who made the order) (MHA s 243(e)).

For more information please refer to the [CAMHS Admission to CAMHS Ward 5A Procedure](#).

The Right to Make a Complaint about Mental Health Services

Every person has the right to complain about the provision of health care.

Information about making complaints must be made available at admission and on request. This information must include the process for making a complaint both internally, to the health service provider, and externally, to an independent statutory authority.

CAMHS services must have an internal complaints procedure that is reviewed regularly and revised as necessary (MHA ss 308(1)(a), 308(1)(b)).

Children and their families should be encouraged to first raise their concerns with the relevant CAMHS service to allow for the dispute to be resolved at a local level and in a timely manner.

Where the dispute cannot be resolved by the relevant CAMHS service, the person making the complaint must be advised to contact the Health and Disability Services Complaints Office (HaDSCO).

For further information, please refer to the following resources:

- [MHA](#) ss 308 – 347
- [CPG](#) Chapter 1 (1.8), Chapter 7
- WA Health Consumer feedback [webpage](#)
- WA Health [Complaints Management](#) policy
- CAHS [Consumer Feedback Management](#) policy

The Right to Access a Mental Health Advocate

A child who fits the definition of an identified person, pursuant to section 348 of the MHA, has a right to access a mental health advocate. The meaning of identified person is outlined in the [definitions](#) section of this policy document.

When a child requests to be contacted by an advocate, CAMHS staff must notify the Chief Mental Health Advocate within 24 hours of the request being made (MHA s 356(3)).

For further information, please refer to the following resources:

- [MHA](#) ss 348 - 378
- [CPG](#) Chapter 1 (1.16), Chapter 8
- CAMHS [Mental Health Act Legal Orders and Notifications](#) policy

The Right to Access Medical Records and Other Documents

A child or their family has a right to inspect and be given a copy of any documents held by CAMHS that pertain to the care or treatment of that child (MHA s 248). This includes the whole or any part of the child's medical record.

A child or their family may be refused access to these records if:

- a psychiatrist reasonably believes that disclosure of the requested information would pose a significant risk to the health or safety of the child or to the safety of another person (MHA s 249(1)(a)(i)); or
- a psychiatrist reasonably believes that disclosure of the requested information would pose a significant risk of serious harm to the child or to another person (MHA s 249(1)(a)(ii)); or
- disclosure of the requested information would reveal personal information about an individual that is not the child (MHA s 249(1)(b)(i)); or
- disclosure of the requested information would reveal information of a confidential nature that was obtained in confidence (MHA s 249(1)(b)(ii)).

For further information, please refer to the following resources:

- [MHA](#) ss 247 - 251
- [CPG](#) Chapter 1 (1.6)
- CAHS [Confidentiality, Disclosure and Transmission of Health Information](#) policy
- CAMHS [Mental Health Act Legal Orders and Notifications](#) policy

The Right of a Voluntary Patient to Seek an Admission

A child or their family has the right to request an admission to CAMHS Ward 5A (MHA s 302(2)).

Although the child or their family have a right to request admission, a child can only be admitted to CAMHS Ward 5A by a medical practitioner (MHA s 255) and this admission must be confirmed by a psychiatrist (MHA s 256(1)). Confirmation is not required by a psychiatrist if the medical practitioner admitting the patient is a psychiatrist.

For further information, please refer to the following resources:

- [MHA](#) ss 255 – 257, 302
- [CPG](#) Chapter 1 (1.4)

- [CAMHS Admission to CAMHS Ward 5A Procedure](#)

The Right to a Further Opinion

Involuntary patients have the right to request a further opinion if they are not satisfied with the treatment being provided (MHA s 182(2)). This right also extends to a mentally impaired accused who is detained at CAMHS Ward 5A (MHA s 177(b)). This request can be made orally or in writing to the child's psychiatrist or to the Chief Psychiatrist (MHA s 182(2)). The request must be documented in the child's medical record (MHA s 182(3)).

This request can be made by:

- the child (irrespective of whether they have capacity to consent);
- the child's family (where the child does not have capacity to consent);
- the child's nominated person;
- the child's carer; or
- a close family member of the child (MHA s 182(1)).

For further information, please refer to the following resources:

- [CPG](#) Chapter 1 (1.13)
- CAMHS [Provision for Further Opinions for Voluntary Patients](#) guideline

The Right to be Involved in Treatment, Support and Discharge Plans

Involuntary patients have a right to be involved in the preparation and review of their treatment, support and discharge plans (TDSP) (MHA s 188(1)(a)). This right also extends to a mentally impaired accused detained at CAMHS Ward 5A (MHA s 185(b)).

The TDSP must outline the child's treatment and support for:

- the duration of their involuntary status; and
- the period following discharge from their involuntary status.

In CAMHS, TDSP's are referred to as Management Plans.

A copy of the completed Management Plan must be given to the child and their family, carer, nominated person or close family member where applicable.

The child's involvement must occur irrespective of whether they have capacity to consent to the implementation of the plan and irrespective of whether the plan can be implemented without their consent (MHA ss 188(1)(a)(i), 188(1)(a)(ii)).

It is the responsibility of the patient's treating psychiatrist to ensure that the child and their family are involved in this planning (MHA ss 188(1)(a), 188(1)(b), 188(1)(c)). The treating psychiatrist must also involve the child's nominated person, carer or close family member where applicable (MHA ss 188(1)(d), 188(1)(e), 188(1)(f)). If the

treating psychiatrist is unable to involve any of the aforementioned people, this outcome must be documented along with any attempts at involvement made by the psychiatrist.

The preparation, review and revision of TDSP's are a prescribed requirement for CAMHS under the MHA (MHA s 422(c)). An application to issue CAMHS with a compliance notice can be made to the Mental Health Tribunal (MHA s 424). This application can be made by the child, their family or any other person who, in the opinion of the Mental Health Tribunal, has sufficient interest in the matter (MHA s 424).

For further information, please refer to the following resources:

- [MHA](#) ss 185 – 188
- [CPG](#) Chapter 1 (1.14), Chapter 4 (Addendum 1(3) and Addendum 2(e))
- CAMHS [Clinical Assessment](#) policy
- CAMHS [Clinical Handover](#) guideline

The Right to a Review from the Mental Health Tribunal

An application can be made to the Mental Health Tribunal to review a decision made under the MHA that affected a child's rights under the act (MHA s 434(1)). This application can be made by a child who is an involuntary patient, their family or any other person who, in the opinion of the Mental Health Tribunal, has sufficient interest in the matter (MHA s 434(2)).

Once the review has been completed the Mental Health Tribunal may make any orders and give any directions that they consider appropriate (MHA s 436). This may involve upholding or revoking the treatment decision.

For further information, please refer to the following resources:

- MHA ss 434 – 436
- CPG Chapter 1 (1.17, 1.18, 1.19, 1.20), Chapter 9

Right to be Provided with Copies of Orders

The treating psychiatrist must record any MHA orders on the appropriate form. All MHA forms must be filed in the child's medical record and a copy must be provided to:

- the child; and
- the child's family.

There are circumstances where these forms may also be provided to:

- the child's carer;
- a close family member that does not have parental responsibility for the child;
- the child's nominated person; or
- other support person.

The circumstances where this is permissible depends on the particular order that was made. It is the responsibility of the treating psychiatrist to:

- determine who may receive a copy of the form; and
- ensure a copy is provided to the appropriate people.

The MHA gives the child and family the right without restriction to copies of any forms, apart from the addendum to Form 1A.

For further information, please refer to the following resources:

- [CPG](#) Chapter 3 (3.5.12)
- CAHS [Confidentiality, Disclosure and Transmission of Health Information](#) policy
- CAMHS [Mental Health Act Legal Orders and Notifications](#) policy
- [SSCD and MHA Forms](#)

The Right to Not be Ill-treated

CAMHS staff must not ill-treat or wilfully neglect any voluntary or involuntary mental health patient including a person referred for an examination by a psychiatrist or a mentally impaired accused detained in an authorised hospital (MHA s 253). The penalty for doing so may result in a fine amounting to \$5000.

Patient Rights that may be Invoked in Inpatient CAMHS Services Only

The Right to Request an Interview with a Psychiatrist

A child who is admitted to CAMHS Ward 5A as an inpatient, either voluntary or involuntary, has the right to request an interview with a psychiatrist at any time during their admission (MHA s 260(1)).

The person in charge of CAMHS Ward 5A must ensure that the request is complied with within a reasonable time frame (MHA s 260(2)(a)) and that the request is documented in the child's medical record (MHA s 260(2)(b)).

A psychiatrist may refuse the request if the child has a history of making these requests (MHA s 260(4)(a)) or the psychiatrist is satisfied that the child is acting unreasonably in making the request (MHA s 260(4)(b)).

If the psychiatrist refuses the request, this refusal must be documented in the child's medical record along with their reasons for refusal (MHA s 260(5)). A copy of this documentation must also be provided to the child (MHA s 260(5)).

For further information, please refer to the following resources:

- [MHA](#) s 260
- [CPG](#) Chapter 1 (1.11)

The Right to Use or Store Personal Possessions

A child who is admitted to CAMHS Ward 5A as an inpatient, either voluntary or involuntary, is entitled to:

- have their personal possessions stored in a secure facility (MHA s 259(3)(a)); and
- use their personal possessions when requested (MHA s 259(3)(b)).

This right also extends to a mentally impaired accused who is detained at CAMHS Ward 5A (MHA s 258(c)).

This right does not apply to possessions that:

- may pose a risk of harm to the patient or another person (MHA s 259(4)); or
- are not appropriate to store at an authorised hospital (MHA s 259(5)).

CAMHS Ward 5A is not required to store personal possessions indefinitely, the circumstances in which they may be disposed of are outlined in section 259(6) of the MHA.

For further information, please refer to the following resources:

- [MHA s 259](#)
- [CPG Chapter 1 \(1.10\)](#)
- PCH [Searching Patients and their Belongings](#) policy
- [CAMHS Admission to CAMHS Ward 5A Procedure](#)

The Right to Communication

A child who is admitted to CAMHS Ward 5A as an inpatient, either voluntary or involuntary, has the right to free and lawful communication (MHA s 261(2)). This right also extends to a mentally impaired accused who is detained at CAMHS Ward 5A (MHA s 258(c)). Examples of lawful communication are outlined in section 261(3) of the MHA. The child must be given reasonable privacy when exercising their right to lawfully communicate (MHA s 261(3)).

A psychiatrist can make an order prohibiting or limiting an inpatient's right to communicate. The criteria for prohibiting or limiting an inpatient's right to communicate are outlined in sections 262(2), 262(3) and 262(4) of the MHA.

The order must be documented on the approved form (MHA s 262(5)):

- [Form 12C – Restriction on Freedom of Communication](#)

This form must be filed in the child's medical record and a copy must be provided to the child and their family, carer, nominated person or other personal support person (MHA ss 262(6)(a), s 262(6)(b)).

This order must be reviewed by a psychiatrist every 24 hours to determine if the order should remain in force, be amended or revoked (MHA s 262(7)). Their determination must be documented on the approved form (MHA s 262(8)):

- [Form 12Ci – Attachment Restriction on Freedom of Communication](#)

If the order is not reviewed before the end of the 24-hour period, the order will cease to be in force (MHA s 262(9)).

For further information, please refer to the following resources:

- [MHA](#) ss 261 – 262
- [CPG](#) Chapter 1 (1.12)

Rights Applicable to Patients of Aboriginal or Torres Strait Islander Descent

Pursuant to the MHA (MHA ss 50, 81), involuntary patients that are of Aboriginal or Torres Strait Islander descent have the right to ask for their assessment or examination to be conducted in collaboration with the following people:

- Aboriginal or Torres Strait Islander mental health workers; and
- significant members of the person's community, including elders and traditional healers.

This request must be fulfilled to the extent that it is practicable and appropriate to do so.

Related CAHS internal policies, procedures and guidelines
CAHS Confidentiality, Disclosure and Transmission of Health Information Policy
CAHS Consumer Feedback Management Policy
CAHS Language Services Policy
CAMHS Admission to CAMHS Ward 5A Procedure
CAMHS Clinical Assessment Policy
CAMHS Clinical Handover Guideline
CAMHS Mental Health Act Legal Orders and Notifications Policy
CAMHS Provision for Further Opinions for Voluntary Patients Guideline
CAMHS Right to Communication and Receiving Visitors Policy
PCH Searching Patients and their Belongings Policy
References and related external legislation, policies, and guidelines
Clinicians' Practice Guide to the Mental Health Act 2014
Mental Health Act 2014 (WA)
Mental Health Regulations 2015 (WA)
National Framework for Recovery-Orientated Mental Health Services
National Standards for Mental Health Services
National Safety and Quality Health Service Standards 2nd Edition
Useful resources (including related forms)
SSCD and MHA Forms
Charter of Mental Health Care Principles

This document can be made available in alternative formats on request.

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Healthy kids, healthy communities

Compassion

Excellence

Collaboration

Accountability

Equity

Respect

Neonatology | Community Health | Mental Health | Perth Children's Hospital